SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Pembrolizumab + carboplatin/paclitaxel → AC

Triple-negative early breast cancer · before and after surgery · NCCP 00857 · version 3 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This pathway combines immunotherapy, chemotherapy and surgery to reduce recurrence risk in eligible high-risk early triple-negative breast cancer.

Schedule
Cycles 1–4: pembrolizumab and carboplatin on Day 1; paclitaxel on Days 1, 8 and 15. Cycles 5–8: pembrolizumab + doxorubicin/cyclophosphamide (AC) on Day 1 every 21 days. Surgery is followed by pembrolizumab when appropriate.

1. GENERIC SACT RISKS / DISCUSSION POINTS

- ☐ Purpose, expected benefit and treatment plan explained
- ☐ Treatment may be delayed, modified or stopped because of toxicity
- ☐ Unexpected, rare or serious adverse effects can occur
- ☐ Supportive medicines and urgent contact advice discussed
- ☐ Pregnancy avoidance / contraception / fertility discussed where relevant
- ☐ Bone-marrow suppression / infection risk
- ☐ Anaemia / fatigue and low platelets / bleeding
- ☐ Nausea / vomiting and bowel disturbance
- ☐ Mouth soreness / mucositis and skin changes
- ☐ Allergic / infusion reaction and other organ-specific toxicity

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Pembrolizumab
IMMUNOTHERAPY

- ☐ Immune inflammation of bowel, lungs, liver, kidneys or skin
- ☐ Hormone-gland problems, sometimes permanent
- ☐ Rare serious heart or neurological inflammation
- ☐ Infusion reaction
- ☐ Effects can first appear after treatment ends

Carboplatin
PLATINUM CHEMOTHERAPY

- ☐ Low blood counts, infection and bleeding
- ☐ Nausea, vomiting and fatigue
- ☐ Allergic reaction, sometimes after previous exposure
- ☐ Kidney or electrolyte changes

Paclitaxel
TAXANE CHEMOTHERAPY

- ☐ Low blood counts and infection
- ☐ Numbness / tingling, sometimes lasting
- ☐ Hair loss, muscle or joint pain
- ☐ Infusion reaction
- ☐ Mouth, bowel, skin and nail effects

Doxorubicin
ANTHRACYCLINE CHEMOTHERAPY

- ☐ Low counts, infection and mouth ulcers
- ☐ Nausea, hair loss and fatigue
- ☐ Heart damage, sometimes delayed
- ☐ Severe tissue injury if the infusion leaks
- ☐ Rare later blood cancers

Cyclophosphamide
ALKYLATING CHEMOTHERAPY

- ☐ Low counts and infection
- ☐ Nausea, vomiting and hair loss
- ☐ Bladder irritation / bleeding
- ☐ Fertility effects and menopause
- ☐ Rare later blood cancers

Discuss the treatment aim, alternatives, expected benefit, serious and lasting risks, fertility/pregnancy precautions, the planned monitoring and who to contact. Review the full longitudinal pathway and confirm which conditional treatments apply.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00857 v3.

SACTCheck v0.83.7
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